Hordeolum, Chalazia, and Cysts

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Hordeolum

- **External hordeolum**
  - **Infection of a Zeiss gland**
- **Internal hordeolum**
  - **Infection of a meibomian gland**

- ***Staphylococcus aureus* is the infectious agent in 90-95% of cases of hordeolum**

- **Abscess (“Bag of pus”)**

Hordeolum

Subjective

- **Patients usually complain of a localized painful swelling on one eyelid**
- **In some cases, the complaint may start as a generalized edema and erythema of the lid that later becomes localized.**
- **A history of similar problems is common.**
- **Constitutional signs and symptoms are inconsistent with a hordeolum diagnosis**

Hordeolum

Epidemiology:

Found more frequently in persons who have

- **Diabetes**
- **Other debilitating illness**
- **Chronic blepharitis**
- **Seborrhea**
- **High serum lipids (High lipid levels increase the blockage rate of sebaceous glands, but lowering of serum lipid levels in these patients has not decreased frequency of recurrence)**

External Hordeolum

- Localized, raised, red bump near lid margin
- Tender to touch
- “Points” to skin side of lid

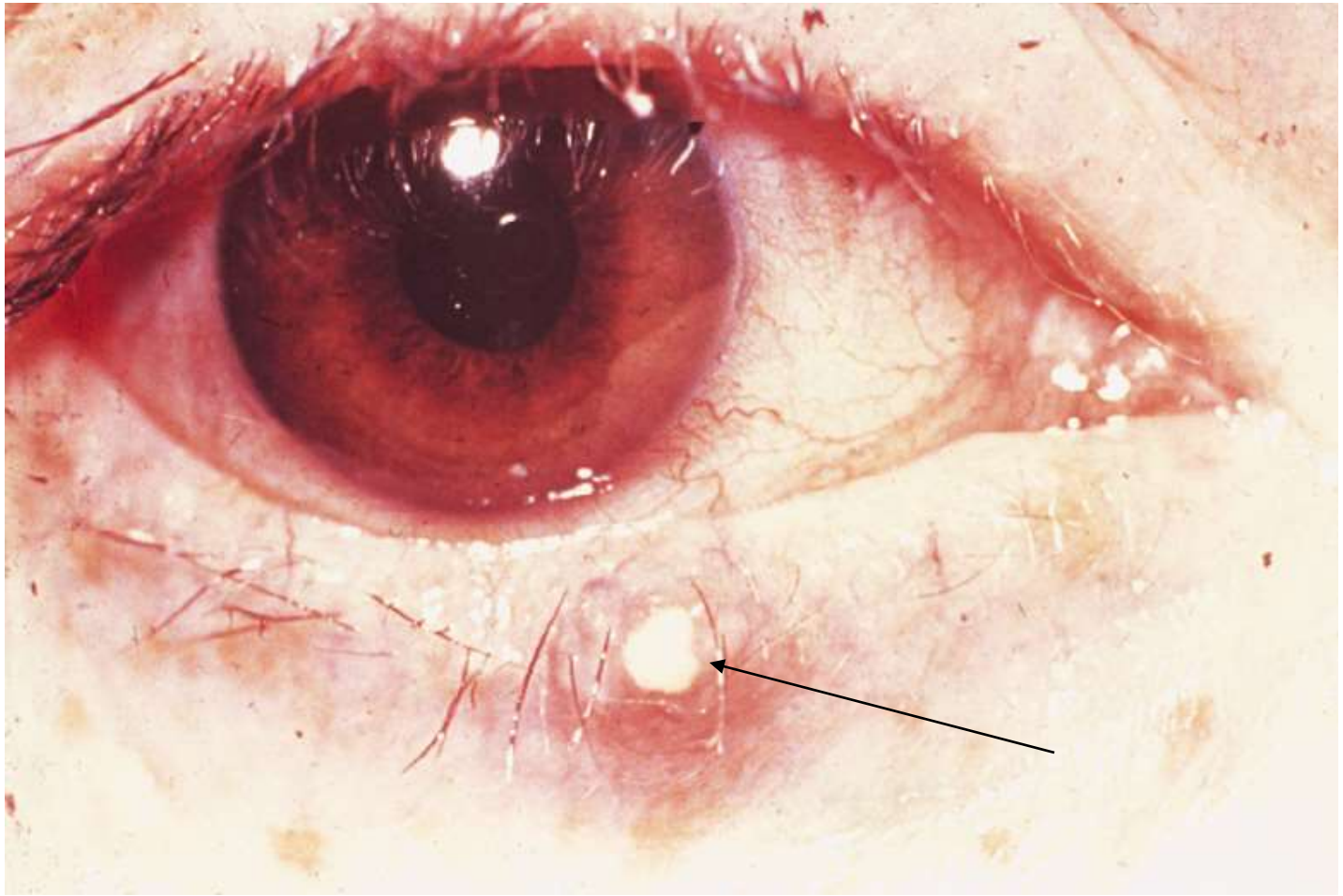


External Hordeolum



External Hordeolum

Hordeolum associated with staphylococcal blepharitis



External Hordeolum

- **Localized infection**





External Hordeolum

- No treatment; usually are self-limited
  - Most eventually point and drain by themselves
  - Discharge anteriorly, close to the lashes
- Warm soaks (qid for 15 min) are the mainstays of treatment
- Topical antibiotics may be used for recurrent lesions and for those that are actively draining
- Epilation of eyelash associated with infected follicle
  - May enhance drainage of pus

Internal Hordeolum

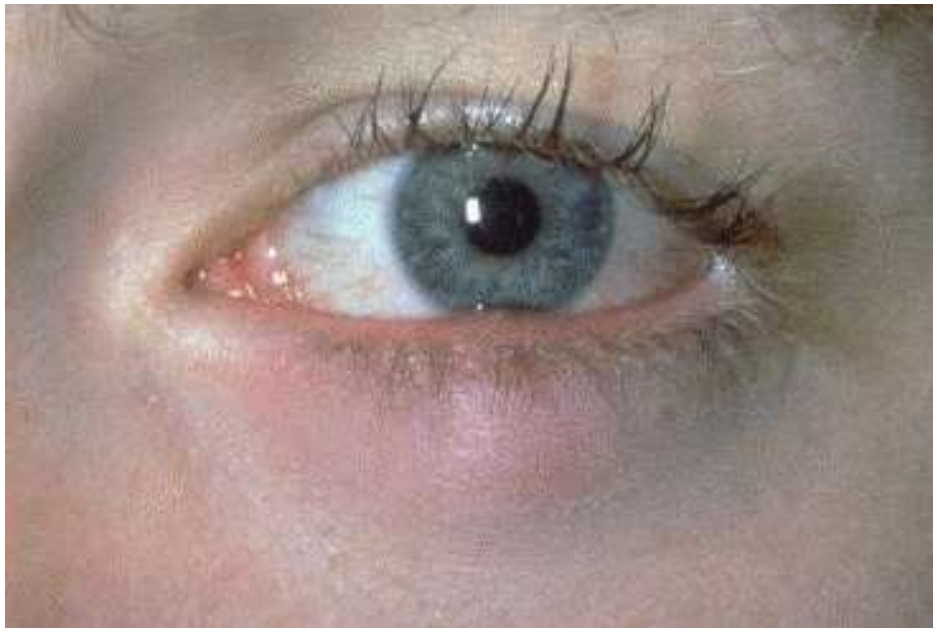
- Localized, red, raised bump
- Tender to the touch
- Points to the conjunctival side of the lid

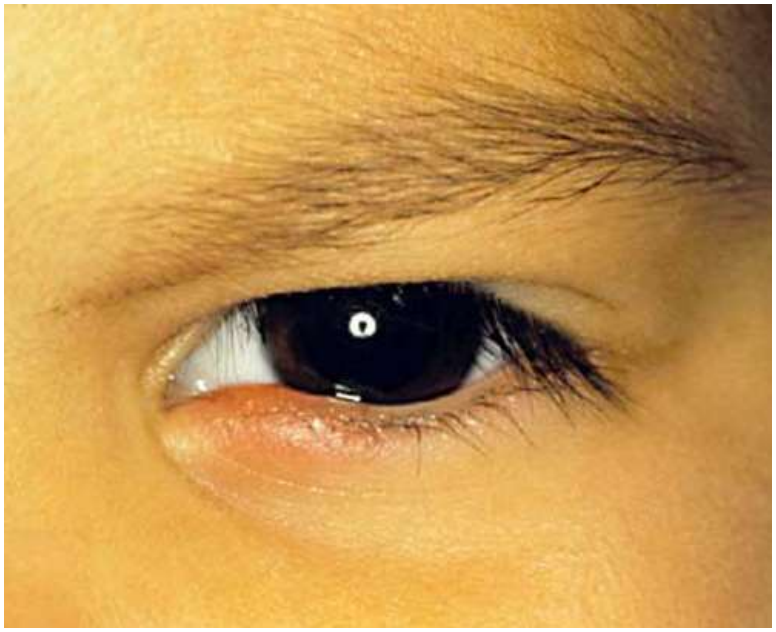


Internal Hordeolum



Internal Hordeolum









Internal Hordeolum







Internal Hordeolum

- **Large internal hordeolum discharging anteriorly through skin**



- **Infection can spread from the gland to the lid connective tissue → preseptal cellulitis**



Internal Hordeolum

- Usually are self-limited
- Most eventually point and drain by themselves.
- Warm soaks (qid for 15 min) are the mainstays of treatment.
- Topical antibiotics may be used for recurrent lesions and for those that are actively draining
- Surgical drainage of pointed lesions can speed the healing process
- Systemic antibiotics are indicated only when inflammation has spread beyond the immediate area of the hordeolum

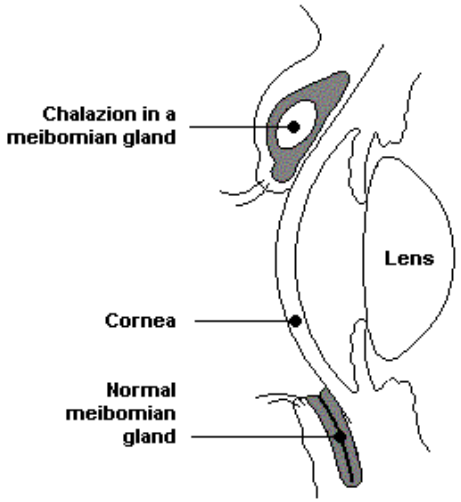
Internal Hordeolum

Drainage of a hordeolum

- Stab incision at the site of pointing using an 18-gauge needle or a #11 blade
- External incisions lead to scarring, so making external eyelid incisions or punctures is inadvisable, unless the hordeolum already is pointing externally
- A large abscess may have multiple pockets and require multiple stabs.
- Internal incisions should be made vertically to minimize the area of cornea swept by a scar during blinking
- Leave the incision open with a clean margin

Chalazion

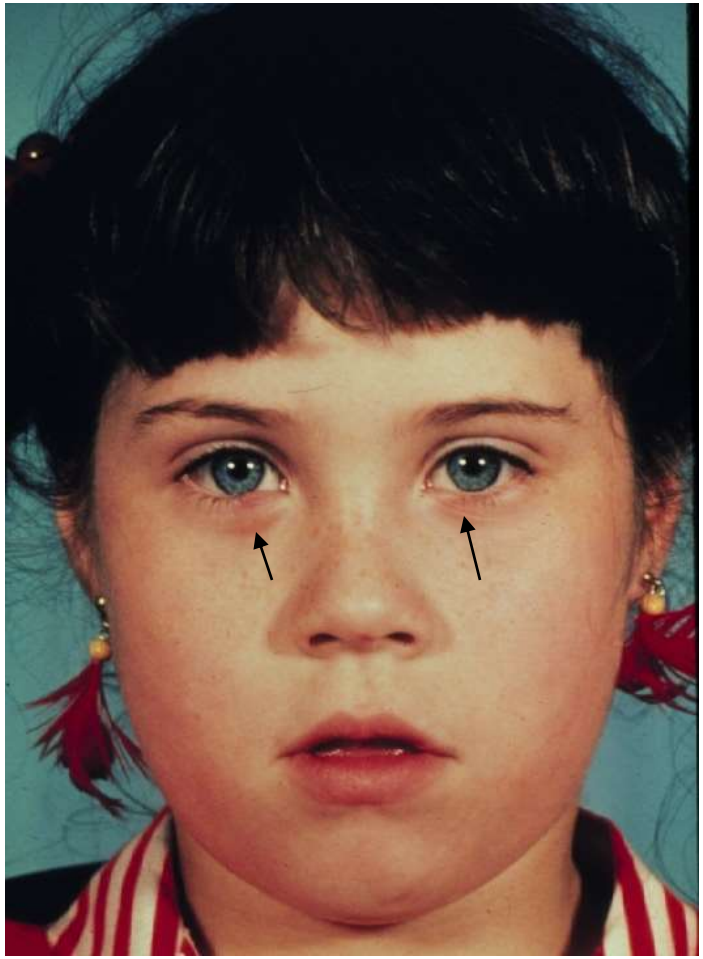
- AKA meibomian cyst
- Lipogranulomatous infiltration of meibomian gland
- May follow an internal hordeolum
- Signs
  - Firm nodule within tarsal plate
  - Non-tender, non-inflamed
- Some refer to a internal hordeolum as an acute chalazion



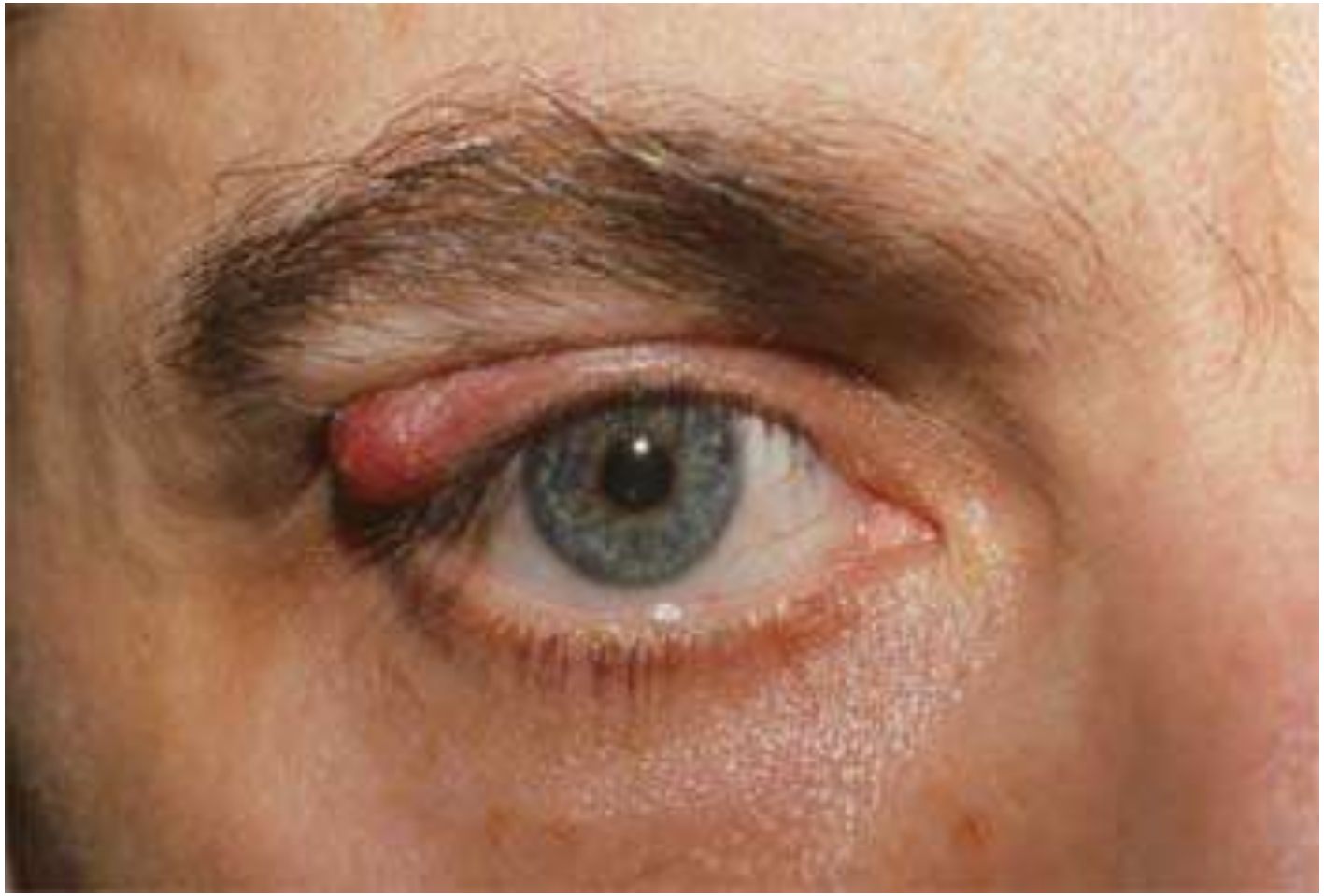
Chalazion



Chalazia



Chalazion



Chalazion





Chalazion



Chalazion



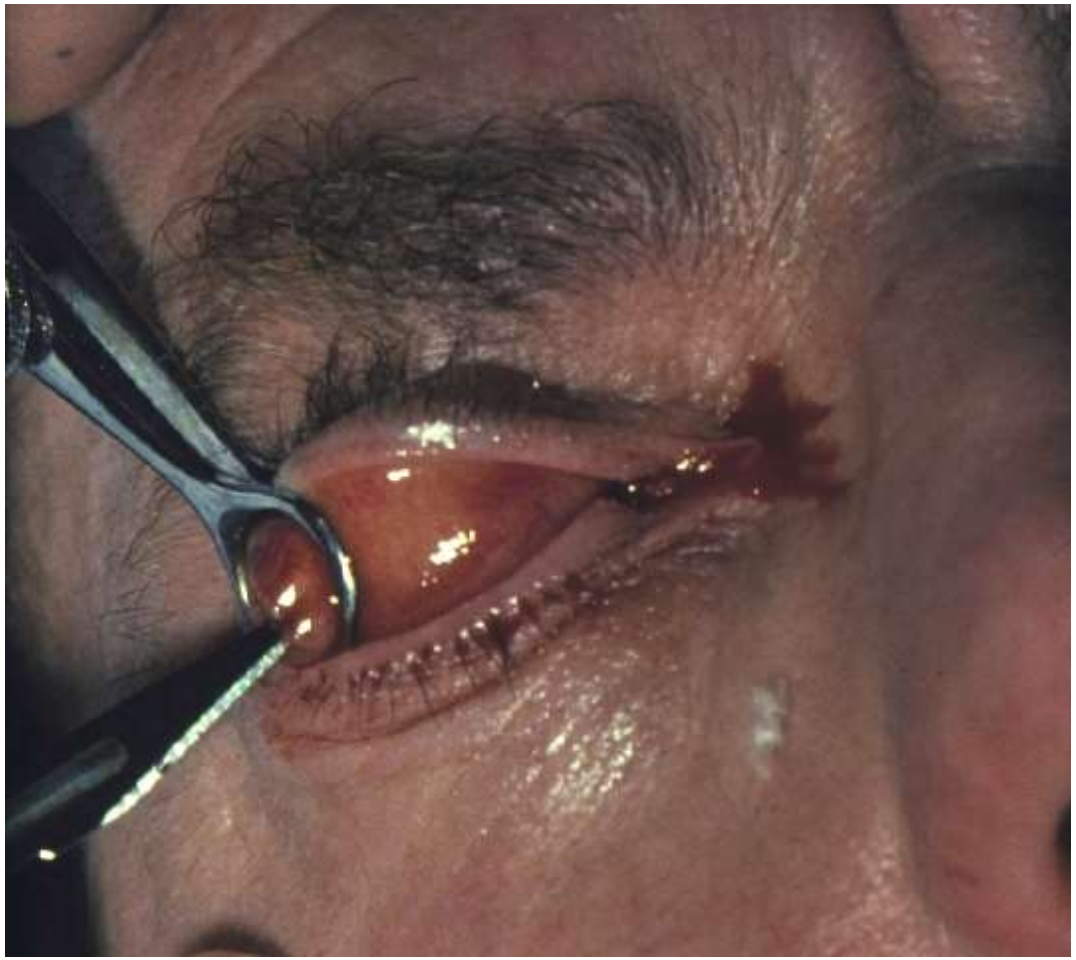
Chalazion

- Warm compresses
  - 10-15 minutes
  - At least qid but q2h better
  - Work best if lesion is recent
- Steroid injection into lesion
  - 0.1-0.2 ml triamcinolone injected through conjunctiva
- Incision and curettage
  - Local anesthetic
  - Chalazion clamp
  - Incision from conjunctival side
  - Curettage - scrape out material
  - Topical antibiotic ointment - X 2 days





Incision of Chalazion



Cyst of Moll

- AKA suderiferous cyst or apocrine hidrocystoma
- Arises from a blocked sweat-secreting gland associated with the cilia (gland of Moll)



Cyst of Moll

- Dome shaped
- Non-tender
- No inflammation
- Translucent
- Fluid-filled
- Anterior lid margin

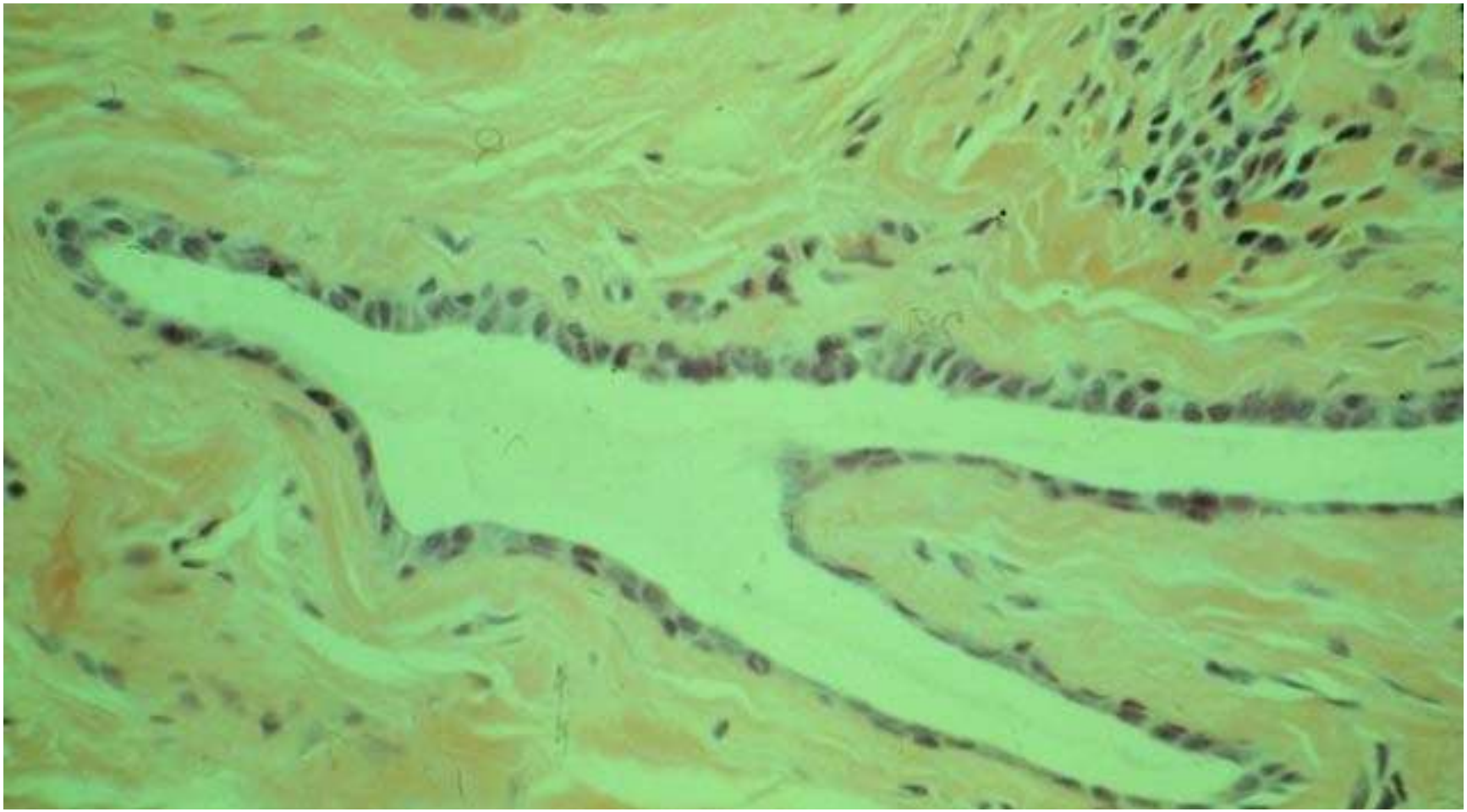


Cyst of Moll



Image courtesy of Shaina Hewitt, PUCO Class of 2019

Cyst of Moll



Cyst of Moll

Treatment

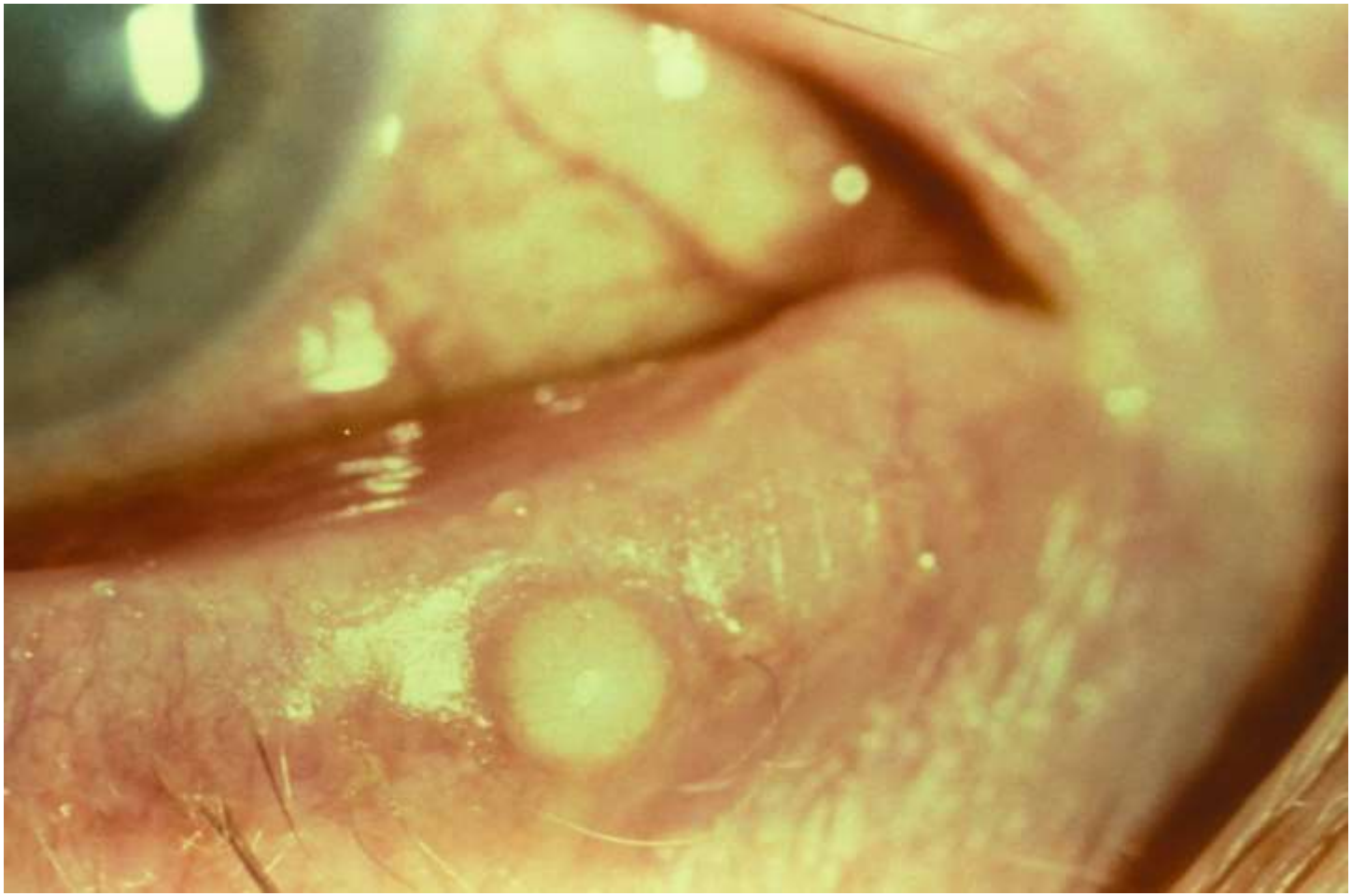
- **Primarily a cosmetic concern**
- **Can aspirate with a fine needle but recurrence is common**
- **Excision with removal of the sac to avoid recurrence**



Cyst of Zeis

- Arises from a blocked oil-secreting gland associated with the cilia (gland of Zeis)
- Contains oily secretions and is less translucent than a cyst of Moll

Cyst of Zeis



Sebaceous Cyst

- Forms when an oil gland in the skin is blocked
- Can be found on any part of the body
- Yellowish-white lumps on the skin
  - Have a central punctum
- If squeezed, exude a stringy, cheesy white material

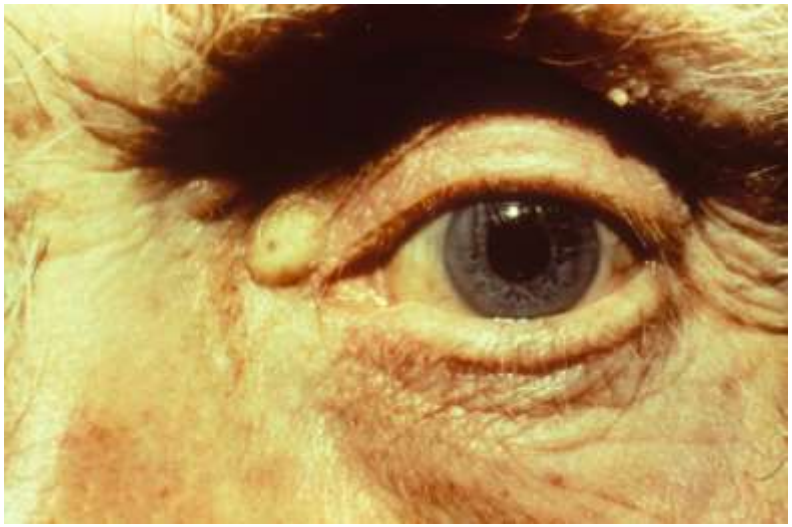


Sebaceous Cyst



Sebaceous Cyst







Differentiate sebaceous cyst from xanthelasma (aka xanthoma)…



Sebaceous cyst



Xanthelasma

Sebaceous Cyst

- Primarily a cosmetic problem
- The total sac of the cyst must be removed
- If the cheesy material is just squeezed out, the cyst will reform
- In the case of an infected cyst, an oral antibiotic should be prescribed

Sebaceous Cyst

- **Good hygiene may prevent the formation of sebaceous cysts**
  - **Some individuals are predisposed to development of these cysts, so even careful hygiene does not prevent them**

THANK YOU!